Outlook

FW: Submission of Sentinel Event Memorandum for Record – Request for Review and Action - Protected Disclosure

From Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>

Date Wed 21-May-25 14:18

To Barney, Sue M. (IHS/PHX) <Sue.Barney@ihs.gov>; Golin, Rachel (IHS/PHX/AO) <Rachel.Golin@ihs.gov>; Hicks, Kenny (IHS/PHX) <Kenny.Hicks@ihs.gov>; Reidhead, Ty (IHS/PHX) <Ty.Reidhead@ihs.gov>; Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>; Weahkee, Michael (IHS/PHX) <Michael.Weahkee@ihs.gov>; Brady, Margaret (IHS/PHX) <Margaret.Brady@ihs.gov>; Elliott, Holly M (IHS/PHX) <Holly.Elliott@ihs.gov>

Cc OIG Hotline <OIG.Hotline@oig.hhs.gov>; Berg, Bill (CMS/OPOLE) <William.Berg@cms.hhs.gov>

3 attachments (3 MB)

Memorandum Sentinel Event 05_21_25 Ward Lund and Pope_Signed.pdf; Protected Disclosure May 25, 2025_Signed.pdf; Memorandum to GB 052125_Signed.pdf;

Dear Members of the Governing Body,

Please find attached a formal memorandum titled "Memorandum to GB 052125_Signed." This memorandum outlines significant gaps in PIMC's current Safety Event Reporting Policy, including the absence of a clearly defined sentinel event structure, misalignment with federal and accreditation standards, and potential risks to patient safety and organizational compliance.

These concerns have been previously elevated to Executive Leadership through multiple channels—including memoranda, protected disclosures, and direct correspondence—but have not resulted in corrective action. The attached document consolidates these communications, highlights relevant Joint Commission, CMS, and IHS policy requirements, and provides actionable recommendations for resolution.

I respectfully request that the Governing Body review the findings and direct appropriate policy development or revision to ensure regulatory compliance, safeguard patient care standards, and support staff in effective event response.

Please do not hesitate to contact me with any questions or if additional documentation is needed. I remain committed to supporting this effort in my capacity as Policy and Procedure Nurse Consultant.

Respectfully,
Hon. Kevin D. Turnage, MSN Ed., RN, NHDP-BC, NPD-BC, AMB-BC, NE-BC
Policy and Procedure Nurse Consultant
Phoenix Indian Medical Center
Kevin.Turnage@ihs.gov | (602) 647-9126

From: Turnage, Kevin D. (IHS/PHX)
Sent: Wednesday, May 21, 2025 09:03
To: Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>; Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>; Aird, Stephanie (IHS/PHX/PIMC) <Stephanie.Aird@ihs.gov>; Dill, Kathy A. (IHS/PHX) <Kathy.Dill@ihs.gov>; Hixson, Naomi H. (IHS/PHX) <Naomi.Hixson@ihs.gov>; Tillman, Laura (IHS/PHX) <Laura.Tillman@ihs.gov>; Hosetosavit, Harriet (IHS/PHX) <Harriet.Hosetosavit@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Holiday,

Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>; Todecheenie, Sheila E. (IHS/PHX) <Sheila.Todecheenie@ihs.gov>; Downing, Mark (IHS/PHX/PIMC) <Mark.Downing2@ihs.gov>
Cc: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: RE: Submission of Sentinel Event Memorandum for Record – Request for Review and Action - Protected Disclosure

Dear Ms. Ward Lund and Mr. Pope,

I hope this message finds you well.

Please find attached two (2) memoranda One is "titled Protected Disclosure May 25, 2025_Signed" and one is titled "Alignment of Sentinel Event Policy with CMS and Joint Commission Standards." This document outlines specific concerns regarding the absence of sentinel event language in our current Safety Event Reporting Policy, the implications for compliance, and recommendations for aligning our internal policies with federal and accreditation expectations.

Given the continued designation of IHS as a high-risk organization by the GAO, and the Chief Quality Officer's acknowledgment that the current policy was modeled directly after Department of Defense guidance, I believe this issue warrants prompt and serious attention. As outlined in the memorandum, the current approach does not fully meet the regulatory or accreditation standards applicable to our environment.

I welcome further discussion on how best to proceed and remain available to assist in the policy development or revision process as needed.

Thank you for your time and consideration.

Respectfully,

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
Policy and Procedure Nurse Consultant, COR II,
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From: Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>
Sent: Wednesday, May 14, 2025 16:34
To: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>; Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>; Aird, Stephanie (IHS/PHX/PIMC) <Stephanie.Aird@ihs.gov>; Dill, Kathy A. (IHS/PHX) <Kathy.Dill@ihs.gov>; Hixson, Naomi H. (IHS/PHX) <Naomi.Hixson@ihs.gov>; Tillman, Laura (IHS/PHX) <Laura.Tillman@ihs.gov>; Hosetosavit, Harriet (IHS/PHX) <Harriet.Hosetosavit@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>; Todecheenie, Sheila E. (IHS/PHX) <Sheila.Todecheenie@ihs.gov>; Downing, Mark (IHS/PHX/PIMC) <Mark.Downing2@ihs.gov>
Cc: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: RE: Submission of Sentinel Event Memorandum for Record – Request for Review and Action

Good Afternoon,

Thank you Mr. Pope - I concur with your assessment and recommendations.

Sincerely,

Deb

Debra Ward Lund, RN, MSN
Chief Executive Officer
Phoenix Indian Medical Center
Indian Health Service

From: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Sent: Wednesday, May 14, 2025 3:52 PM
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>; Ward Lund, Debra L. (IHS/PHX)

<Debra.Wardlund@ihs.gov>; Aird, Stephanie (IHS/PHX/PIMC) <Stephanie.Aird@ihs.gov>; Dill, Kathy A. (IHS/PHX) <Kathy.Dill@ihs.gov>; Hixson, Naomi H. (IHS/PHX) <Naomi.Hixson@ihs.gov>; Tillman, Laura (IHS/PHX) <Laura.Tillman@ihs.gov>; Hosetosavit, Harriet (IHS/PHX) <Harriet.Hosetosavit@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>; Todecheenie, Sheila E. (IHS/PHX) <Sheila.Todecheenie@ihs.gov>; Downing, Mark (IHS/PHX/PIMC) <Mark.Downing2@ihs.gov>
Cc: McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: RE: Submission of Sentinel Event Memorandum for Record – Request for Review and Action

Thank you Mr. Turnage,

As stated before, the Safety Event Reporting Policy covers the things you are addressing here, it is a comprehensive policy addressing reportable events, which covers what TJC refers to as sentinel events. Is there something specific about the Safety Event Reporting Policy you are concerned with? Is there something it's missing or needs better clarification? Or is there a concern over the title? Your claims are very broad and from my review of the policy, your compliance concerns are properly covered/addressed in this policy. Please be more specific if you would like anything added to or removed from this policy.

The term sentinel event is used by TJC but not by CMS. In DoD policy they use the term "Reportable Event," so I shamelessly stole that term from their policy. I do not share your concerns in using this terminology. Our policy does define the terms Reportable Event and Safety Event and the policy does define the requirements and timeline for conducting an RCA per TJC and IHS HQ, it also describes and directs the use of the I-STAR system. It includes detailed attachments and flowcharts to help communicate and clarify these processes.

I agree that we need to adjust other policies to correctly point to the Safety Event Reporting policy. As you have mentioned, I ask that you please work with those policy owners to assist in those adjustments. I can also add references in this policy to IHS policy and TJC standards around safety event reporting and RCA processes.

I agree that we could benefit from increased staff awareness of this policy, I have shared it in multiple settings, and while assisting with reportable events and suspected reportable events. There has also been targeted training and sharing of policy details with I-STAR handlers, who are responsible for the investigation of safety events.

In short, I fail to see a compliance gap regarding sentinel event policy. But I do see areas to improve and thank you for your efforts to help us make those improvements. If you experience any pushback when working with policy owners to make the needed changes, please inform their leadership and address those concerns in your report to the Quality Council.

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ
Chief Quality Officer
Indian Health Service
Department of Health and Human Services
Phoenix Service Unit

Office: (602)581-6563
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From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Wednesday, May 14, 2025 9:02 AM
To: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>; Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>; Aird, Stephanie (IHS/PHX/PIMC) <Stephanie.Aird@ihs.gov>; Dill, Kathy A. (IHS/PHX) <Kathy.Dill@ihs.gov>; Hixson, Naomi H. (IHS/PHX) <Naomi.Hixson@ihs.gov>; Tillman, Laura (IHS/PHX) <Laura.Tillman@ihs.gov>; Hosetosavit, Harriet (IHS/PHX) <Harriet.Hosetosavit@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>; Todecheenie, Sheila E. (IHS/PHX)

<Sheila.Todecheenie@ihs.gov>; Downing, Mark (IHS/PHX/PIMC) <Mark.Downing2@ihs.gov>
Subject: Re: Submission of Sentinel Event Memorandum for Record – Request for Review and Action

Dear Mr. Pope,

BLUF (Bottom Line Up Front):

The Phoenix Service Unit (PSU) lacks a formal Sentinel Event Policy, creating a critical compliance gap with The Joint Commission (TJC) and Centers for Medicare & Medicaid Services (CMS) requirements. This absence risks citations, fines, and potential loss of accreditation. Additionally, the mismatch in terminology between "Sentinel Events" and "Reportable Events," gaps in the I-STAR reporting system, and inconsistent medical device event tracking further compound these compliance challenges. While I can support policy owners in aligning their documents, my position lacks the formal authority to mandate these updates, making immediate leadership intervention essential.

DETAILS

Thank you for responding to my May 8, 2025, memorandum regarding the Sentinel Event Policy. I appreciate your efforts to clarify the use of the term **"Reportable Events"** within the Safety Event Reporting Policy. However, after a comprehensive review of the relevant policies, I have identified several critical compliance gaps that must be addressed to fully align with **The Joint Commission (TJC)** and **Centers for Medicare & Medicaid Services (CMS)** requirements for Sentinel Event management.

1. Sentinel Event Policy Gap (Regulatory Non-Compliance)
Issue: There is a critical gap in the absence of a formal Sentinel Event Policy at the Phoenix Service Unit (PSU), as noted in my initial memorandum. This gap violates TJC standards, which mandate that accredited healthcare organizations maintain a comprehensive Sentinel Event Policy.
Regulatory References:

- **TJC Standards** require hospitals to identify sentinel events, conduct Root Cause Analyses (RCA) within 45 days, and implement corrective actions to prevent recurrence.

- **42 CFR §482.21** requires hospitals to track adverse patient events as part of their Quality Assessment and Performance Improvement (QAPI) program.
  Impact: Non-compliance may result in citations, fines, or potential loss of accreditation by TJC and CMS, directly impacting patient safety and the quality of care provided at PSU.
- The policy can be titled "**Reportable Event**" or any other title that covers the identification and management of sentinel events; however, it **must** include a definition of **Sentinel Event** that aligns with both **The Joint Commission (TJC)** and **Centers for Medicare & Medicaid Services (CMS)** standards.

2. Misalignment of Terminology for Reportable Events
Issue: While the PSU uses the term **"Reportable Event"** as a broader category encompassing **"Sentinel Events"** under the Safety Event Reporting Policy, this terminological mismatch creates confusion and inconsistency in policy application and event reporting.
Recommendation: Policies should clearly align terminology to reflect both TJC and CMS definitions, ensuring consistency in documentation, training, and event response.

3. I-STAR Reporting System Issues

Issue: The **I-STAR** system, the designated platform for tracking reportable events, including sentinel events, lacks a dedicated Sentinel Event Policy. This gap can undermine the effectiveness of the RCA process and overall event management.
Recommendation: Immediate development of a standalone Sentinel Event Policy to integrate with the I-STAR system, ensuring comprehensive event tracking, timely RCA completion, and corrective action planning.

4. Medical Device Reporting and Management Compliance
Issue: The **Medical Device Failure Policy** and **Procedure for Recall of Potentially Hazardous Products** lacks integration with the Sentinel Event Framework, potentially leading to gaps in the tracking and reporting of adverse events related to medical devices.
Regulatory References:

- The **Safe Medical Devices Act (SMDA) of 1990** requires the timely reporting of device-related injuries or deaths.
  Recommendation: Harmonize these policies to ensure medical device failures are clearly defined as potential sentinel events where appropriate, aligning with federal requirements for medical device safety.

5. Medical Staff Bylaws and Professional Review
Issue: The **Medical Staff Bylaws** include references to sentinel events as criteria for professional reviews and corrective actions, but without a clear guiding policy, this process lacks consistency and clarity.
Recommendation: Update the bylaws to reflect the new Sentinel Event Policy once established and ensure all corrective actions are aligned with both CMS and TJC standards.

Next Steps for Compliance:

- **Develop a Standalone Sentinel Event Policy:**
  - Clearly define sentinel events in alignment with TJC and CMS requirements.
  - Establish explicit RCA and Corrective Action Plan (CAP) timelines, including the required 45-day deadline.
  - Define roles and responsibilities for leadership, including the Governing Body, in Sentinel Event oversight.
- **Update Existing Related Policies:**
  - Ensure all related policies (e.g., Safety Event Reporting, Medical Device Failure, Risk Management) correctly reference this new policy and include integrated processes for comprehensive oversight.
- **Implement Staff Training and Awareness Programs:**
  - Develop training materials to educate staff on the importance of proper Sentinel Event identification, reporting, and response.

Why a Dedicated Sentinel Event Policy is Best Practice

While TJC does not specifically require a single "Sentinel Event Policy," it strongly encourages

healthcare organizations to develop clear, structured policies and procedures that address each of these elements to ensure consistent and effective responses to sentinel events. This approach is considered best practice and significantly reduces regulatory risk.

Authority to Mandate Policy Updates:
I understand that you have requested my assistance in working with policy owners to correct these gaps. However, as you have previously noted, my Position Description (PD) does not grant me the authority to require policy owners to make these critical updates. This responsibility appropriately resides with the leadership team, as effective policy alignment requires coordinated action at that level.

Nonetheless, I remain fully committed to supporting this effort within the scope of my role. I am prepared to provide technical guidance, policy language support, and compliance resources to ensure these changes are effectively implemented.

Very Respectfully,

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
Policy and Procedure Nurse Consultant
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Cell: 602-647-9126

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Furthermore, please note that I am not authorized to obligate the government for any work, service, or product. Do not perform any work that will require payment without a valid contract issued by a government Contracting Officer (CO). Changes to a contract can only be made by the Contracting Officer. Contractors are reminded to promptly notify the Contracting Officer of any conduct that may be regarded as a change to the contract in accordance with FAR 52.243-7. The Contracting Officer's Representative (COR) is not authorized to make any changes to the terms, conditions, or scope of the contract.

From: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Sent: Friday, May 9, 2025 07:50
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>; Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>; Aird, Stephanie (IHS/PHX/PIMC) <Stephanie.Aird@ihs.gov>; Dill, Kathy A. (IHS/PHX) <Kathy.Dill@ihs.gov>; Hixson, Naomi H. (IHS/PHX) <Naomi.Hixson@ihs.gov>; Tillman, Laura (IHS/PHX) <Laura.Tillman@ihs.gov>; Hosetosavit, Harriet (IHS/PHX) <Harriet.Hosetosavit@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>; Todecheenie, Sheila E. (IHS/PHX) <Sheila.Todecheenie@ihs.gov>; Downing, Mark (IHS/PHX/PIMC) <Mark.Downing2@ihs.gov>
Subject: RE: Submission of Sentinel Event Memorandum for Record – Request for Review and Action

Mr. Turnage,

Thanks for your email and for recognizing these gaps. We do have a policy addressing sentinel events. It is called the "Safety Event Reporting" Policy. The term "Sentinel Event" is used by the Joint Commission. At the PSU, we use the term "Reportable Event," which includes events that meet the Joint Commission's definition of sentinel event.

Thank you for recognizing that other policies point to a policy or policy name that does not exist. Will you please help those policy owners to correct those policies to point to the correct policy?

Thanks,

Clint

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ

Chief Quality Officer

Indian Health Service

Department of Health and Human Services

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From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Friday, May 9, 2025 6:00 AM
To: Ward Lund, Debra L. (IHS/PHX) <Debra.Wardlund@ihs.gov>; Aird, Stephanie (IHS/PHX/PIMC) <Stephanie.Aird@ihs.gov>; Dill, Kathy A. (IHS/PHX) <Kathy.Dill@ihs.gov>; Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>; Hixson, Naomi H. (IHS/PHX) <Naomi.Hixson@ihs.gov>; Tillman, Laura (IHS/PHX) <Laura.Tillman@ihs.gov>; Hosetosavit, Harriet (IHS/PHX) <Harriet.Hosetosavit@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>; Todecheenie, Sheila E. (IHS/PHX) <Sheila.Todecheenie@ihs.gov>; Downing, Mark (IHS/PHX/PIMC) <Mark.Downing2@ihs.gov>
Subject: Submission of Sentinel Event Memorandum for Record – Request for Review and Action

Dear Executive Leadership Team,

Pursuant to the quality of care requirements established under **25 U.S.C. § 1661**, which mandates the Indian Health Service (IHS) to provide comprehensive, high-quality healthcare to American Indian and Alaska Native (AI/AN) populations, I am formally submitting the attached **Sentinel Event Memorandum for Record** for your review and action.

This memorandum addresses critical gaps identified in the absence of a comprehensive Sentinel Event Policy, as required by **42 C.F.R. § 482.21** and the **2025 Joint Commission National Patient Safety Goals (NPSG)**. These regulations mandate that hospitals establish robust systems for identifying, analyzing, and preventing sentinel events, including the timely completion of root cause analyses (RCAs) to protect patient safety and ensure continuous quality improvement.

Additionally, this submission is made with full awareness of the protections afforded under the **Federal Whistleblower Protection Act (5 U.S.C. § 2302)**, which safeguards federal employees against retaliation for disclosing information concerning violations of laws, regulations, gross mismanagement, gross waste of funds, and dangers to public health and safety.

If there are any existing policies, guidelines, or documents that address these sentinel event requirements, or that may provide further clarity on these matters, please share them for reference.

Your prompt attention to this matter is critical to maintaining compliance with federal quality standards and ensuring the highest levels of patient care at Phoenix Indian Medical Center.

Additionally, the absence of a comprehensive Sentinel Event Policy may impact the following related policies, which will also need to be updated to ensure alignment once the new policy is finalized:

- 16495443 - Healthcare Risk Management and Patient Safety
- 14936122 - After Hours Hospital Oversite
- 11284239 - Clinical Alarm Safety and Management
- 155589764 - Code Grey: Abusive or Violent Conduct
- 17992512 - Code Stork Response
- 16766145 - Medical Staff Bylaws
- 14059333 - Medication Events
- 17697960 - Point of Care Testing Quality Management
- 16969672 - Safety Event Reporting
- 13676963 - Sentinel Events - Departmental - Lab
- 17020676 - Compliance Plan

Once the Sentinel Event Policy is finalized, I will work with the appropriate teams to ensure these related policies are updated accordingly.

Upon request, I am prepared to provide a detailed justification memorandum to further support this submission.

Thank you for your attention and consideration.

Respectfully,

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
Policy and Procedure Nurse Consultant, COR II
Phoenix Indian Medical Center
Cell: 602-647-9126

This report is submitted in good faith based on my understanding of the relevant legal and regulatory requirements. I acknowledge that these requirements may be interpreted differently by others, and I remain available for further discussion or clarification as necessary.

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